

Cancellation or Alteration of Response

Standard:

Establish direction for cancelling or altering an initial response to a request for service.

Purpose:

To give all providers in the ATCEMS System guidance on when they may be able to alter or cancel an initial response based on patient or scene presentation.

Application:

1. Resources will be initially dispatched to a 9-1-1 request for service based on the currently approved Medical Priority Dispatch (MPD) standards.
2. After assessing the patient(s) and making a determination of needed resources any on-scene Credentialed Provider may modify or cancel the response mode of any other System Provider not already on-scene.
 - a. Prior to scene arrival, EMS specialty units staffed with at least one provider at the $\geq$ PL5 level or Office of the Chief Medical Officer ($\geq$ PL7) providers may, at their discretion, replace an assigned ambulance on priority 4 or 5 calls, except PSYC4F, to respond and initiate patient assessment/care then determine the need for a transport unit or disposition options.
 - i. If the call priority changes to 1-3, then an ambulance will be assigned to respond.
 - ii. When replacing an assigned ambulance, a specialty unit should proceed to the call in a non-emergency response mode.
 - b. A credentialed Integrated Services – Collaborative Care Communications Center (C4) provider may navigate the incident and cancel responding units on calls dispatched to the C4 following appropriate MPDS triage.
3. If cancelled, responders may, at their discretion, reduce their response to non-lights and sirens ("Code 1") and continue to provide other assistance deemed appropriate by their organization or department. This does not apply to responses for responsibilities other than patient care (scene safety, fluids, etc).
4. At the discretion of the Integrated Services Medical Director and the Integrated Services Division Chief, responses may be modified for ongoing incidents of inappropriate 9-1-1 use.